

Massachusetts Eye and Ear Audiology Department

NAME: DANIEL MAY **DOB:** 10/11/1989
AGE: 35 **SEX:** M
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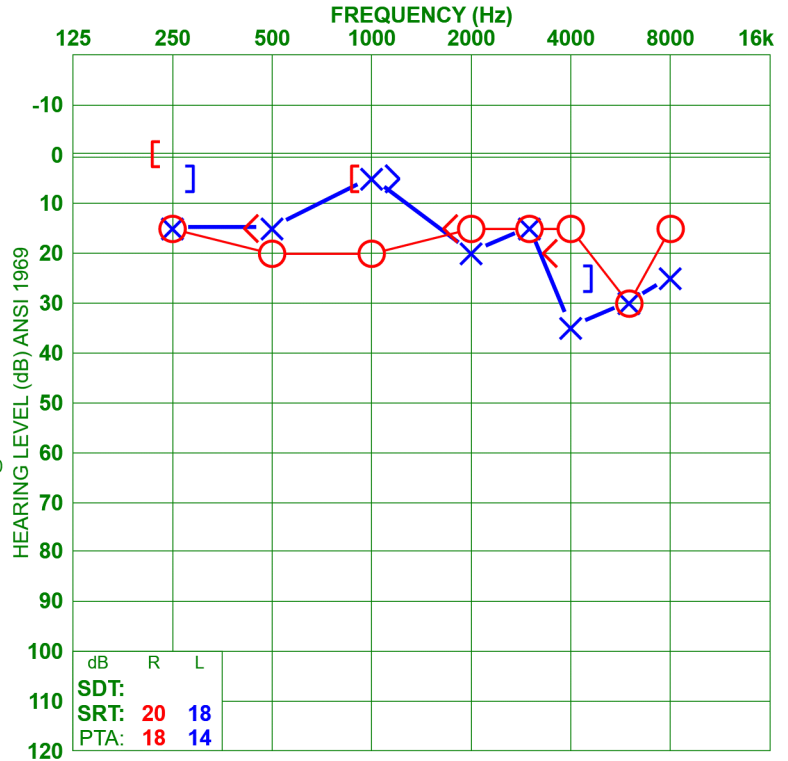
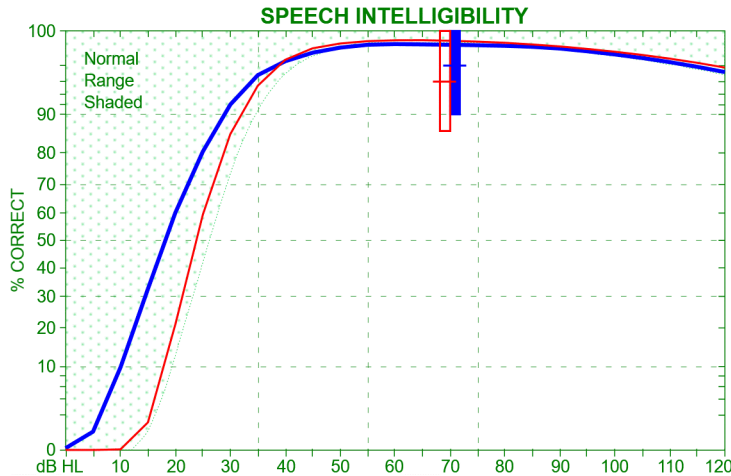
AUDIOLOGIST:

Electronically signed by Anna M. Keenan, Au.D., CCC-A

TEST SITE: 243 CHARLES ST., BOSTON, MA 02114

DATE: 10/04/2025 **TIME:** 09:30 **STATUS:** FINAL

COPY

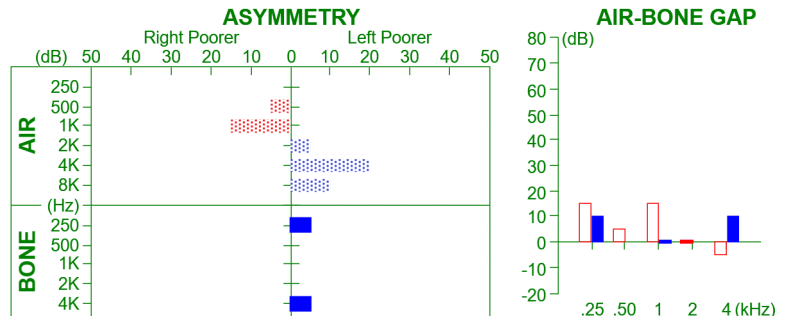


SPEECH TESTING			
RIGHT		LEFT	
LIST	dBHL Score	LIST	dBHL Score
W22 3A	70 96%	W22 4A	70 98%

SYMBOLS	RIGHT	LEFT
Speech Intelligibility Index Prediction		
Speech Score 95%CD		
Air Conduction		
AC Masked		
Bone Conduction		
BC Masked		
Response at Limit		
No Response		
Acoustic Reflex Ipsilateral Probe		
Acoustic Reflex Contralateral Probe		

TYMPANOMETRY		
FEATURE	RIGHT	LEFT
VOLUME	1.0 ml	0.7 ml
PRESSURE	-5 daPa	-181 daPa
MOBILITY	2.2 ml	0.6 ml
SHAPE	Normal	Normal

REFLEX DECAY		
FREQUENCY	RIGHT	LEFT
500 Hz		
1000 Hz		
2000 Hz		
4000 Hz		



REASON FOR TEST

Eustachian tube dysfunction

HISTORY

Patient is seen for suspected eustachian tube dysfunction. He reports aural fullness and popping bilaterally, left ear worse. Patient has a history of sinus surgeries. He denies additional otologic complaints.

HEARING MEASUREMENT

TDH and insert earphones were used for testing. Ipsilateral acoustic reflexes to broadband stimulation are absent in the right ear.

IMPRESSIONS

Tympanometry results are consistent with normal middle ear pressure and hypermobility in the right ear, and negative middle ear pressure with normal mobility in the left ear. Conductive overlay is noted at 250 and 1000 Hz in the right ear. Absence of acoustic reflex suggests the conductive components are of middle ear origin. The patient may experience communication difficulties in adverse listening situations, and is expected to benefit from the use of good communication strategies.

CONSULTATION

Results were reviewed with the patient.

RECOMMENDATIONS

-Monitor hearing in conjunction with medical management.